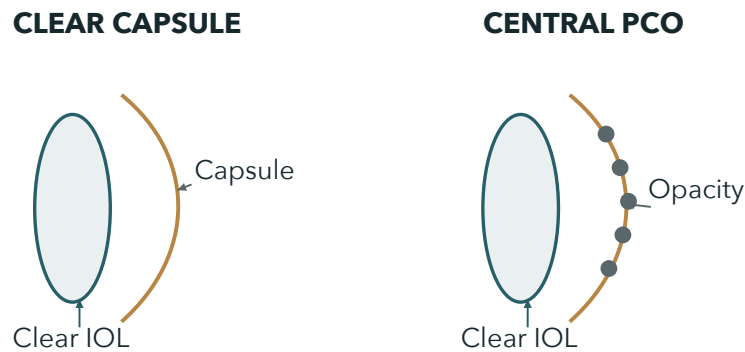


Lens lids and paediatric eye

CLINICALS | Match functional impairment and examination findings to the next useful action.

Cataract and blur after surgery

Removed lens, retained capsule



Original schematic. PCO clouds the retained capsule behind the IOL; the removed lens has not regrown.

Clinical decision

Finding	Action
Painless blur/glare with lens opacity	Confirm cataract explains symptoms: refraction, slit lamp and fundus; look for coexisting disease.
Function impaired despite optical correction	Discuss surgery using function, preferences, benefit and risk. No fixed acuity threshold or maturity requirement.
Mild impact; prefers to defer	Spectacles, lighting and monitoring help function; they do not remove cataract.
Late painless blur; central capsule opacity	Symptomatic PCO: consider Nd:YAG capsulotomy after assessing the rest of the eye.

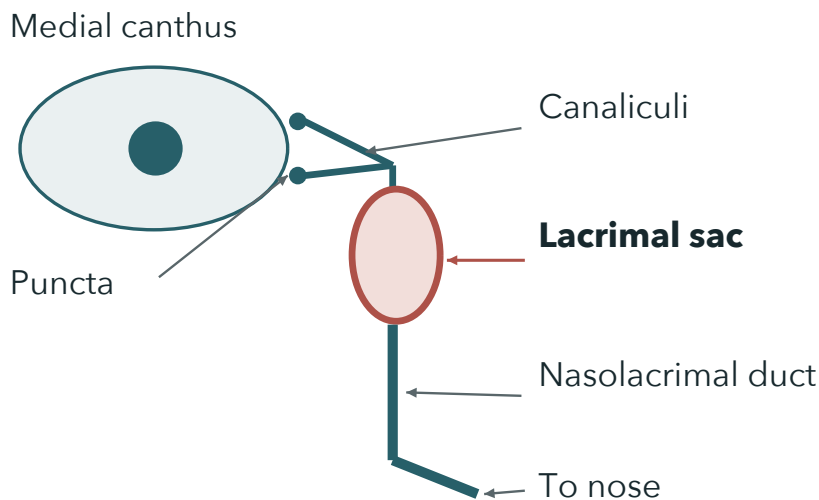
Before and after surgery

- Axial-length biometry and corneal keratometry guide IOL power. Prior refractive surgery complicates calculation.
- Retinal disease can limit the result; spectacles may remain necessary.
- New postoperative pain, redness or falling vision → immediate eye-service assessment for infection, pressure or retinal complications.

References NICE NG77: Cataract assessment and surgery | Moorfields: Nd:YAG treatment for PCO | AAO EyeWiki: Postoperative endophthalmitis

Lids and lacrimal sac: locate the lesion

Tear drainage and the medial swelling



Original tear-drainage schematic. Sac infection produces swelling below the medial canthus.

Common patterns

Finding	Initial clinical approach
Tender lash-root pustule	External hordeolum: warm compresses; review spreading or persistent inflammation.
Usually painless tarsal nodule	Chalazion: compresses/lid care; specialist procedure if persistent or significant. Routine antibiotics do not resolve a sterile granuloma.
Same-site recurrence, lash loss or lid distortion	Evert lid; specialist assessment/tissue diagnosis for malignancy, including sebaceous carcinoma.
Tender medial swelling, tears, pus reflux and fever	Dacryocystitis: systemic antibiotics, ophthalmic assessment; admit/IV if severe or spreading. Check orbital extension.

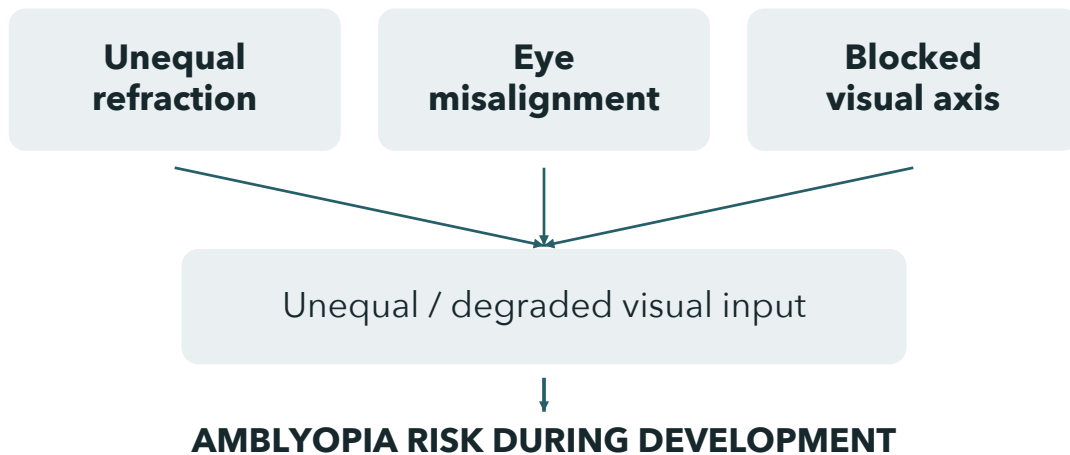
Surface protection in lid malposition

Entropion rubs lashes on cornea; ectropion exposes conjunctiva and displaces the punctum. In facial palsy, check closure, sensation and staining. Use lubrication, night ointment and eyelid-closure protection. Epithelial defect, pain or reduced vision needs urgent review.

References AAO EyeWiki: Chalazion and recurrent lesions | College of Optometrists: Acute dacryocystitis | College of Optometrists: Facial palsy eye care, 2026

Childhood alignment and amblyopia

Treat the source of unequal visual input



Correct the cause + rehabilitate vision

Original schematic. Optical clarity, alignment and brain visual development are related but require separate assessment.

What the examination changes

Finding	High-yield action
Unequal acuity / fixation preference	Check structures and cycloplegic refraction before diagnosing amblyopia.
Hyperopia; esotropia improves in correction	Full-time prescribed hyperopic correction; review acuity and residual deviation.
Amblyopia persists after optical correction	Specialist patching or penalisation of better eye; continue glasses and monitor both eyes.
Epicanthal folds; normal reflex/cover tests	Possible pseudostrabismus. Persistent concern needs re-examination.

Cover-test essentials

- Cover-uncover: observe the uncovered eye refixate to detect a manifest deviation. Alternating cover breaks fusion and reveals total deviation.
- Measure each eye separately; the better eye can mask unilateral loss.
- Treat visual-axis deprivation promptly. Alignment surgery does not replace amblyopia therapy.

References AAPOS: Amblyopia | AAPOS: Strabismus | AAPOS: Accommodative esotropia

Leukocoria is a finding, not a diagnosis

Act on an abnormal reflex

- 1 White or absent/asymmetric red reflex → urgent paediatric ophthalmic assessment.
- 2 New squint + lost red reflex → immediate referral. Squint with headache, vomiting or ataxia → immediate acute paediatric assessment.
- 3 Specialist dilated examination/EUA and ultrasound locate the lesion. Suspected retinoblastoma → ocular oncology and MRI orbits/brain.

Localise the white reflex

Cause	Useful discriminating clue
Retinoblastoma	Retinal tumour, often calcified; negative family history does not exclude it.
Infantile cataract	Lens opacity; urgent assessment to protect visual development.
Persistent fetal vasculature	Often unilateral, small globe and retrolental stalk.
Coats disease	Abnormal retinal vessels and heavy exudation, usually unilateral.
Retinopathy of prematurity	Prematurity with abnormal retinal vascular development; scheduled screening.

Retinoblastoma investigations

Suspected retinoblastoma: diagnosis uses examination and imaging. Avoid routine intraocular needle biopsy because it can spread tumour. MRI assesses optic nerve, orbit and brain; ultrasound/MRI generally replace routine CT.

Family follow-through

Stage confirmed disease; arrange RB1 genetic assessment and fellow-eye examination. Priorities: survival, then eye preservation and useful vision.

Paediatric clues that change the pathway

A watering infant is not always a blocked duct

Pattern	Action
Tearing/crusting; clear comfortable cornea	Likely congenital nasolacrimal obstruction: massage; drops only for significant discharge. Persistent/severe disease may need probing.
Photophobia; enlarged or hazy cornea	Urgently exclude childhood glaucoma.
Fever + painful medial swelling; dacryocystocele	Emergency assessment. Systemic antibiotics if infected; nasal extension can impair breathing.

Neurocutaneous clues

- Periocular/forehead port-wine birthmark: assess for Sturge-Weber-associated glaucoma; arrange follow-up from infancy.
- NF1: regular ophthalmic surveillance for optic pathway glioma. New acuity/colour/field loss or proptosis needs reassessment. Lisch nodules do not themselves impair vision.
- Dense infantile cataract or pupil-blocking ptosis: urgent assessment for deprivation amblyopia and visual rehabilitation.

Core examination record

- 1 Acuity/fixation in each eye; refraction when indicated.
- 2 Red reflex, cover testing, movements and pupils.
- 3 Corneal clarity/size, lid closure, fundus and birth/family history.

Follow-up has a purpose

Review visual development after glasses, patching or surgery. A newly paralytic squint requires urgent neurological assessment.